OPERATIONAL EXCELLENCE · WHAT THE RESEARCH SAYS

When the right action became the default, more people did it.

The results didn't always follow.

That is what four randomized trials published from 2023 to 2026 show. All of them come from health care.

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TRIAL ONE

What changed in 11 hospitals?

Under usual care, a patient got a palliative care consult only if a doctor remembered to order one.

Then the consult was ordered automatically for every patient aged 65 or older with advanced chronic lung disease (COPD), dementia, or kidney failure. Doctors could cancel it. Palliative care staffing did not increase.

Palliative care relieves symptoms and helps patients and families make treatment decisions during serious illness.

FINDINGS

More consults. Same length of stay.

43.9% of patients got a consult when it was ordered by default, compared with 16.6% under usual care.

3.4 days after admission, on average, the consult came, instead of 4.6 days.

But length of stay, the study's primary goal, did not change. The analysis included 24,065 patients.

TRIAL TWO

Treatment for everyone, unless they say no

At a U.S. academic hospital, 1,000 inpatients who smoked were randomly assigned to treatment only for those who asked, or treatment that started automatically and could be declined.

60%

used quit-smoking medication after discharge when treatment started automatically, compared with 34%.

89%

completed at least one counseling call, compared with 37%.

After one month, 22% had quit, compared with 16%. By six months, the groups were close: 19% and 18%.

WHY DOES THIS HAPPEN?

Accepting a default costs nothing. Leaving it takes a decision.

Many people also read a default as a quiet recommendation from whoever built the system.

So the person who designs a form or an order set decides, often without meaning to, what most users will do.

This is the author's interpretation, not something the trials measured.

DEFAULTS DON'T ALWAYS WORK

If people can skip a default, many will

21%

of surgical patients got the pre-filled pain prescription. Surgeons typed the rest by hand, so the total amount did not change.

12

ICUs tested a pre-set ventilator order. Adherence did not rise significantly, and results varied from unit to unit.

WHAT THE STUDIES SUGGEST

What can leaders do?

1

Ask what happens automatically in your forms and systems when nobody chooses anything.

2

Make the recommended action the default, put it where people actually work, and keep saying no easy.

3

Measure two things: was the default used, and did the outcome change?

> **Which default in your systems decides what your team does today, without anyone noticing?**

SOURCES AND LIMITS

What this evidence can and cannot tell us

All four trials come from health care, and two were run in a single hospital or center.

The palliative care trial used data from 2016 to 2018.

None included organizations in the Arab region. Applying the idea to other sectors is an inference.

Editorial disclaimer: The views and analyses presented in this article represent the author's professional interpretation based on the cited sources and do not necessarily represent the official positions of the organizations mentioned. For the original, complete and most current information, please consult the primary sources and official documents listed in the references.

[1] Courtright, K. R., et al. (2024). Default Palliative Care Consultation for Seriously Ill Hospitalized Patients. *JAMA*, 331(3), 224–232.

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